SACTCHECK · TREATMENT PASSPORT

# Nivolumab · every 2 weeks · previously treated non-small-cell lung cancer (NSCLC)

## A record of what you experience

One cycle per page. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Nivolumab 240 mg intravenous (IV) over 30 minutes OR 600 mg as a subcutaneous injection over 3–5 minutes on Day 1 every 2 weeks. Your team confirms one formulation; the whole visit includes checks and preparation. Continue until progression or unacceptable toxicity, with individual benefit and safety review.

The four pages are reusable recording sheets, not a prescribed four-cycle course. Ask for additional sheets as needed.

**Emergency: 112 / 999.** Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

**Contact oncology immediately** for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

**Insert numbers here**

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

**Insert numbers here**

Ask for medical oncology

EVENINGS / OVERNIGHT

**Your out-of-hours pathway / nearest ED**

Other hospitals: use the numbers your team provides.

Your own hospital / contact instructions:

Hormone replacement alone does not always mean nivolumab must stop. Your team checks symptoms and recovery and reviews any medicines that suppress the immune system before deciding whether treatment can continue or restart.

https://sactcheck.com/patient/00483/

Full patient information, sources and visual timeline. No identifiers are encoded.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

# Nivolumab · every 2 weeks · previously treated NSCLC

Cycle number: _____ Start date: _____ Next review: _____

**2-week monotherapy cycle 1 · confirm tests and symptoms** · copy this sheet for another cycle of the same phase.

### Nivolumab · Immunotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Breathing / cough** | □ Usual breathing | □ New cough or breathlessness: call promptly | □ Symptoms with usual activity: urgent review | □ Breathless at rest: urgent assessment | □ Severe breathing difficulty / collapse: 112/999 |
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/day or at night: call | □ 7+ extra stools/day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Skin / nail changes** | □ None | □ Mild change: report | □ Pain or daily tasks affected: call | □ Spreading infection or blistering: urgent review | □ Severe illness / collapse: emergency help |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

# Nivolumab · every 2 weeks · previously treated NSCLC

Cycle number: _____ Start date: _____ Next review: _____

**2-week monotherapy cycle 2 · confirm tests and symptoms** · copy this sheet for another cycle of the same phase.

### Nivolumab · Immunotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Breathing / cough** | □ Usual breathing | □ New cough or breathlessness: call promptly | □ Symptoms with usual activity: urgent review | □ Breathless at rest: urgent assessment | □ Severe breathing difficulty / collapse: 112/999 |
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/day or at night: call | □ 7+ extra stools/day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Skin / nail changes** | □ None | □ Mild change: report | □ Pain or daily tasks affected: call | □ Spreading infection or blistering: urgent review | □ Severe illness / collapse: emergency help |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

# Nivolumab · every 2 weeks · previously treated NSCLC

Cycle number: _____ Start date: _____ Next review: _____

**2-week monotherapy cycle 3 · confirm tests and symptoms** · copy this sheet for another cycle of the same phase.

### Nivolumab · Immunotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Breathing / cough** | □ Usual breathing | □ New cough or breathlessness: call promptly | □ Symptoms with usual activity: urgent review | □ Breathless at rest: urgent assessment | □ Severe breathing difficulty / collapse: 112/999 |
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/day or at night: call | □ 7+ extra stools/day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Skin / nail changes** | □ None | □ Mild change: report | □ Pain or daily tasks affected: call | □ Spreading infection or blistering: urgent review | □ Severe illness / collapse: emergency help |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

# Nivolumab · every 2 weeks · previously treated NSCLC

Cycle number: _____ Start date: _____ Next review: _____

**2-week monotherapy cycle 4 · confirm tests and symptoms** · copy this sheet for another cycle of the same phase.

### Nivolumab · Immunotherapy

| Symptom | None | Mild | Moderate / call | Severe / urgent | Emergency signs |
|---|---|---|---|---|---|
| **Breathing / cough** | □ Usual breathing | □ New cough or breathlessness: call promptly | □ Symptoms with usual activity: urgent review | □ Breathless at rest: urgent assessment | □ Severe breathing difficulty / collapse: 112/999 |
| **Diarrhoea** | □ Usual pattern | □ Up to 3 extra stools above normal | □ 4–6 extra stools/day or at night: call | □ 7+ extra stools/day or dehydration: urgent review | □ Collapse / severe dehydration: 112/999 |
| **Skin / nail changes** | □ None | □ Mild change: report | □ Pain or daily tasks affected: call | □ Spreading infection or blistering: urgent review | □ Severe illness / collapse: emergency help |
| **Fatigue** | □ Usual energy | □ Tiredness improves with rest | □ Rest does not help; usual tasks affected: call | □ Self-care difficult: urgent review | □ Collapse: 112/999 |

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.